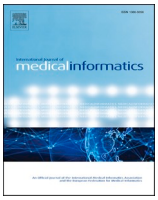
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*Correspondence on "Large language models can support generation of standardized discharge summaries — A retrospective study utilizing ChatGPT-4 and electronic health records"

Dear Editor, we hereby comment on the publication on "Large language models can support generation of standardized discharge summaries – A retrospective study utilizing ChatGPT-4 and electronic health records [1]." The quality of mental discharge summaries (DS) produced by ChatGPT-4 in comparison to those produced by psychiatric residents was the focus of this study. Despite the fact that the results showed notable variations in quality, it is important to address certain content and methodology issues. First, it's possible that the 20 DS sample size is insufficient to make firm judgments regarding generalizability. More data from a bigger sample would be more reliable and aid in determining whether trends seen are consistent across various patient populations. Additionally, the specificity of these prompts is crucial, even though the use of standardized prompts and pseudonymized comments is praiseworthy. AI outcomes could inadvertently be prejudiced if they are not meticulously crafted to represent the intricacy of clinical documentation.

There are issues with the assessment technique as well. While a customized Likert scale with 15 subcategories may cause raters to interpret the data differently, blinded scoring by eight psychiatric doctors promotes rigor. For instance, personal bias may have an impact on subjective judgments. It would be helpful to know how the experiences and backgrounds of raters affected their evaluations. Additionally, a more thorough examination of the type and therapeutic ramifications of these errors would support the findings, even though this study emphasizes the existence of hallucinations in AI-generated summaries. How might the kinds of hallucinations that were documented impact clinical judgment?

Expanding the debate generates a few questions: What specific aspects contribute to the distinct characteristics of human-generated summary ratings, such as language, nuances, or clinical insights, which AI aims to replicate? How do our findings compare to the existing literature on AI in clinical papers, and what conclusions can we draw from these comparisons? Understanding the qualitative elements of both types of DS may bring useful insights for developing AI-generated material.

Future research should focus on improving AI systems to eliminate hallucinations and increase the clinical value of their findings, which could include including feedback loops in which human-generated summary data informs and trains AI. Furthermore, investigating hybrid models that combine human supervision with AI aid in DS production could be a novel approach to increasing both efficiency and quality. Furthermore, it would be beneficial to broaden this research to other specialties and practices, since this could show new problems and potential for AI in clinical records. Finally, it is critical that AI tools complement, rather than replace, human therapists as technology advances.

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Viroj Wiwanitkit: Conceptualization, Methodology, Supervising, Approval for submission

Declaration of competing interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

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